

**PRESENTATION BY THE REGISTRAR, PHARMACISTS COUNCIL OF NIGERIA
(PCN)**

**PHARM. AHMED T. MORA, FPSN, FPCPHARM
ON THE LAWS AND REGULATIONS GOVERNING PHARMACY PRACTICE IN
NIGERIA AND RECIPROCATION**

**AT THE FIRST ANNUAL CONVENTION OF THE NIGERIAN ASSOCIATION OF
PHARMACISTS AND PHARMACEUTICAL SCIENTISTS IN THE AMERICAS
(NAPPSA), HOUSTON, TEXAS, USA
SEPTEMBER 14-16, 2007**

Introduction

It is a pleasure to be here today at the maiden convention of the Nigerian Association of Pharmacists and Pharmaceutical Scientists in the Americas (NAPPSA). When this invitation first came—through phone calls, emails, and a subsequent visit to my office in Abuja by your President, Mr. Nndum Iheme, accompanied by Mr. Jonathan Obiagwu—I realized that Nigerian pharmacists in the USA and across the Americas have not forgotten their motherland.

I congratulate the President, the Board, the Executive Committee, and indeed all members of NAPPSA on this important convention. Please keep the flag flying.

I have been asked to speak on the topic: **“Laws and Regulations Governing Pharmacy Practice in Nigeria and Reciprocation.”**

1.0 Historical Perspective of Pharmacy and Drug Laws in Nigeria

a. Origins of Pharmacy in Nigeria

The practice of pharmacy in Nigeria is closely linked to the arrival of colonial missionaries along the West African coast in the 17th century. Archives reveal that the Portuguese, who came with doctors and nurses to the coastal towns, also brought medicines in the form of mixtures, lotions, and tablets.

By 1887, Mr. Richard Zacheus Bailey had established a drug store along Balogun Street in Lagos. Before then, hospitals and dispensaries had already been opened in Lagos, Warri, Asaba, and Calabar.

With the expansion of British colonial influence in the 19th and 20th centuries, Nigeria was fully integrated into British administration. From this perspective, one can safely argue that modern pharmacy practice in Nigeria is rooted in Great Britain, guided by British ethics of practice, bilateral agreements between sovereign nations, and Memoranda of Understanding under the World Confederation of Pharmacists.

As with all established professions, pharmacy has historically sought to maintain standards in qualification and practice. In Nigeria, this led to the institutionalization of regulatory bodies established by legal instruments.

Today, several regulatory agencies influence pharmacy practice in Nigeria, including:

- **Pharmacists Council of Nigeria (PCN)**
- **National Agency for Food and Drug Administration and Control (NAFDAC)**
- **National Drug Law Enforcement Agency (NDLEA)**
- **Ministerial and Executive Policies** of the Federal Government on health matters (e.g., the Draft Health Act).

b. Brief History of Pharmacy Laws in Nigeria

The earliest laws controlling pharmacy practice date back to **1878**, when the Lagos Pilotage and Harbour Ordinance was enacted to regulate medical treatment and the use of medicines on board ships docked in Lagos Harbour.

Other key milestones include:

- **1881:** Hospital Ordinance.
- **1889:** Ereko Dispensary Rules.
- **1902:** Pharmacy Ordinance No. 8, which regulated the sale and distribution of drugs and poisons (initially restricted to Lagos Colony and a few protectorates). Under this law, dispensers and druggists were examined and certified.
- **1925:** Establishment of the Medical College, Yaba, Lagos, which produced trained dispensers. This prompted the enactment of the Poison and Pharmacy Ordinance, strengthening registration of dispensers, chemists, and druggists.
- **1936:** Poison and Pharmacy Ordinance revised.
- **1958:** Poison and Pharmacy Act (Cap. 152), which amended earlier provisions.

In recent judicial review, the history of pharmacy regulation was summarized in the _____ case: **Nigeria Association of General Practice Pharmacists Employers (NAGPPE) vs. PCN, Suit No. FHC/KD/CP/8/2001.**

In this case, Justice Liman of the Federal High Court, Kaduna (2001) observed:

“Both Counsel did not bother to familiarize themselves with the history of pharmacists and pharmaceutical chemists, when it should have been obvious that the Poison and Pharmacists Act 1958 was partially repealed in 1964, and retained as the Pharmacy Act Cap 26 (1990 as Cap 357 LFN). Section 26(1) of the Pharmacists Council of Nigeria Decree No. 91 of 1992 repealed Cap 357. Therefore, the only law regulating the practice of pharmacy in Nigeria today is **Decree 91 of 1992.**”

2.0 Laws Governing Pharmacy Practice in Nigeria – Repeals and Amendments

It is the desire of every government to provide good healthcare services to its citizens. Under **Chapter II** of the **Constitution of the Federal Republic of Nigeria (1990, Cap 62)**, Section 17(2)(c) provides that:

“The State shall direct its policy towards ensuring that there are adequate medical and healthcare facilities for all persons.”

In line with this constitutional provision, successive governments have enacted legislations on health and pharmacy practice. Key laws include:

2.1 Pharmacists Council of Nigeria (PCN) Decree No. 91 of 1992

- Established the **Pharmacists Council of Nigeria (PCN)**, replacing the former Pharmacy Board of Nigeria (PBN), which was created in 1964.
- Charged with responsibilities including:
 1. Determining the standards of knowledge and skills required for registration as a pharmacist.
 2. Establishing and maintaining registers of persons entitled to practice, and publishing same.
 3. Reviewing and enforcing codes of conduct for pharmacists.
 4. Regulating and controlling pharmacy practice in all its aspects.
- Also established:
 - **Pharmacists Disciplinary Tribunal**
 - **Investigating Panel** (for cases of professional misconduct).

- **Sanctions:** Punishments are limited to removal from register, suspension, or warning. The highest punishment (Section 23(4)) is a fine of ₦1,000 or imprisonment up to 2 years—a grossly inadequate deterrent in modern times.

2.2 Food and Drugs Act, Cap. 150 LFN 1990 (as amended by Decree No. 21 of 1999)

- Regulates the **sale, manufacture, distribution, importation, exportation, and advertisement** of food, drugs, cosmetics, and devices.
- Initially empowered the **Minister of Health** to issue certificates to manufacturers (delegable authority), subject to the presence of a Superintendent Pharmacist.
- Enforcement functions are now carried out by **NAFDAC**, though the Act technically remains valid law.
- **Penalties:** Fine not exceeding ₦1,000 or imprisonment up to 2 years (grossly outdated).

2.3 NAFDAC Decree No. 15 of 1993 (Deemed effective from 1st October 1992)

- Established the **National Agency for Food and Drug Administration and Control (NAFDAC)** as successor to the Food and Drug Administration Department of the FMoH.
- Functions include regulating and controlling the importation, exportation, manufacture, distribution, advertisement, and use of drugs, cosmetics, foods, chemicals, and medical devices.
- NAFDAC is also empowered to inspect premises, raw materials, and enforce compliance.
- **Penalties:** Offenders face fines up to ₦5,000 or imprisonment up to 2 years (again inadequate).

2.4 NDLEA Decree Cap. 253 LFN 1990 (as amended by Decree 15 of 1992)

- Established the **National Drug Law Enforcement Agency (NDLEA)**, with full police powers.

- Empowered to inspect premises dealing in narcotics, check records, and collaborate with other regulatory bodies.
- **Penalties:** Range from 5–25 years imprisonment, with fines up to ₦2 million and possible forfeiture of goods.

2.5 Poisons and Pharmacy Act, Cap. 535 LFN 1990

- Regulates the sale and distribution of poisons and certain classes of drugs.
- Provides for appointment of inspectors authorized to enter premises and enforce compliance.
- **Penalties:** Ridiculously low—₦10 to ₦200 fines or up to 12 months imprisonment.

2.6 Drug and Related Products (Registration, etc.) Decree, 1993 (as amended by Decree 20 of 1999)

- Prohibits the manufacture, importation, advertisement, sale, or distribution of unregistered drugs, cosmetics, and medical devices.
- Enforced by **NAFDAC** through the Drug Registration Committee.
- **Penalties:** Individuals liable to ₦50,000 fine or 2 years imprisonment; corporate bodies liable up to ₦100,000.

2.7 Dangerous Drugs Act, Cap. 91 LFN 1990

- Regulates importation, exportation, manufacture, sale, and use of **opium, cocaine, and other dangerous drugs**.
- Grants regulatory authority to the **President**, Customs, and Excise officers.
- **Penalties:** Fine of ₦200 or imprisonment up to 2 years, including forfeiture of goods.

2.8 National Drug Formulary and Essential Drugs List Act, Cap. 252 LFN 1990

- Provides for a **National Drug Formulary and Essential Drugs List**.
- Prohibits the importation/manufacture of drugs outside the list except with ministerial approval (e.g., for rare diseases).
- Establishes a Review Committee and Secretariat under the FMOH.

- **Penalties:** ₦1,000 fine or imprisonment up to 5 years.

2.9 Counterfeit and Fake Drugs and Unwholesome Processed Foods (Miscellaneous Provisions) Decree No. 25 of 1999

- Prohibits the **manufacture, importation, sale, and distribution** of counterfeit/fake drugs and unwholesome processed foods.
- Provides for the establishment of a **Federal Task Force**, working with the Nigerian Police.
- **Penalties:** Fine not exceeding ₦500,000 or imprisonment between 5–15 years, or both.

3.0 Rules and Regulations Governing Pharmacy Practice

It is important to note that the **subsisting law** regulating and controlling the practice of pharmacy in Nigeria, including the retention of pharmacists' names in the Register, is the **Pharmacists Council of Nigeria (PCN) Act No. 91 of 1992**.

The Act empowers the PCN to make **rules and regulations** to guide its operations. These rules and regulations are compiled, published, and gazetted from time to time. They include the following:

3.1 Key PCN Publications and Guidelines

- i. **Disposal of Poisons Book (2003)** – Used for the disposal and control of dangerous drugs in hospitals, pharmacies, and other health institutions.
- ii. **Code of Ethics for Pharmacists in Nigeria (2003)** – Outlines the moral and professional obligations of pharmacists and prescribes standards of conduct appropriate to the profession.
- iii. **Procedures for Registration (2005)** – Provides detailed procedures for the registration of pharmaceutical premises across various activities regulated by PCN.
- iv. **Guidelines on the Issuance of Patent and Proprietary Medicines Vendors Licences (2003)** – Defines the process for granting licences to vendors. The PCN remains the sole licensing authority.

- v. **Approved Patent Medicine List (3rd ed., 2003)** – Provides an updated list of medicines approved for sale in licensed patent medicine shops nationwide, in line with the **National Drug Policy**.
- vi. **Pharmacy Technicians Training Manual (Rev. ed., 2005)** – Ensures uniformity in curriculum for training pharmacy technicians across Schools of Health Technology in Nigeria.
- vii. **Internship Programme (Rev. ed., 2005)** – Introduces and guides fresh pharmacy graduates into professional practice.
- viii. **Internship Logbook (2005)** – Designed for interns to record weekly work activities during training, signed by preceptors for accountability.
- ix. **Orientation Programme for Patent Medicine Vendors (Rev. ed., 2004)** – Provides structured training and orientation for patent medicine vendors to ensure safe practice, given the limited number of pharmacists nationwide.
- x. **Foreign Pharmacy Graduate Orientation Programme (FPGOP) Manual (2005)** Guides foreign-trained pharmacy graduates on adapting to Nigerian practice standards, laws, ethics, tropical medicine, and public health realities.
- xi. **Pharmacists Council of Nigeria Act No. 91 of 1992** – The principal legislation regulating pharmacy practice in Nigeria.
- xii. **Mandatory Continuing Professional Development (MCPD) Brochure (2005)** - Guides registered pharmacists on procedures for accessing Continuous Professional Development (CPD) programmes, necessary for re-certification.
- xiii. **A Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care in Nigeria (2005)** – Prescribes minimum acceptable standards for pharmacy practice in:
- Manufacturing outfits
 - Hospital pharmacies
 - Community pharmacies
 - Wholesale/importation premises

xiv. **Guidelines for the Establishment of Pharmacy Programmes (2005)** – Provides Nigerian universities with guidelines on establishing faculties of pharmacy and training programmes.

3.2 Gazetted Regulations (2005)

Under the authority of the **PCN Act No. 91 of 1992**, and with the approval of the Honourable Minister of Health, the following regulations were gazetted in 2005:

1. Inspection, Location, and Structure of Pharmaceutical Premises Regulations (2005, No. 79, Vol. 92)

- o Established a **Pharmaceutical Inspectorate Division**.
- o Empowered inspectors to enter premises at reasonable times where pharmaceutical activities are suspected.
- o Prohibited locating pharmaceutical premises in markets, kiosks, or roadside stalls.
- o Prescribed penalties of imprisonment (≥ 6 months) and/or fines (₦50,000).

2. Registration of Pharmaceutical Premises Regulations (2005, No. 81, Vol. 92)

- o Specified requirements for obtaining a premises licence.
- o Restricted retail practice to companies incorporated and wholly owned by registered pharmacists (or partnerships of same).
- o Made the registration of **hospital pharmacies** mandatory.
- o Defined penalties for defaulting premises.

4.0 Extant Laws and Their Position on Reciprocity

As earlier stated, the principal law regulating pharmacy practice in Nigeria is the **Pharmacists Council of Nigeria (PCN) Act No. 91 of 1992**, which formally established the PCN.

This Act provides explicit provisions on:

- **Qualifications for registration** as a pharmacist,

- **Conditions for licensing**, and
- Other related matters governing the profession.

Importantly, the Act confers on the **PCN—exclusively—the statutory authority** to register pharmacists and issue licences. This means that any arrangement or **Memorandum of Understanding (MoU)** that attempts to establish a parallel registration body, even if in collaboration with the PCN, would **contradict the provisions of the law**.

However, the law does not prevent the PCN from:

- Conducting **orientation programmes for foreign-trained pharmacists** in the United States of America (USA), in collaboration with professional associations such as NAPPSA;
- Facilitating **reciprocity arrangements** regarding the retention of Nigerian pharmacists' names on the register, provided such programmes are **not restricted** only to members of any one association (e.g., NAPPSA).

The issue of **reciprocity** has consistently been a key concern for the PCN, particularly in relation to:

1. **Annual licence renewal** for Nigerian pharmacists living abroad, and
2. The **Orientation Programme for Foreign-Trained Pharmacists**, conducted annually (August/September) in rotation across accredited faculties of pharmacy in Nigeria.

This programme is also featured in the PCN's **2007–2011 Strategic Plan of Action**, which was under review by the Council at the time of this presentation.

Furthermore, at its **26th Governing Council meeting** held on **Thursday, August 2, 2007**, the PCN approved an **annual flat fee of \$100 (USD)** for Nigerian pharmacists practicing outside the country to maintain their names on the Register of Pharmacists in Nigeria. Details on implementation were to be issued subsequently.

5.0 Lacuna, Loopholes, and Overlap of Functions among Enforcing Agencies

5.1 Current Situation

Pharmacy practice in Nigeria today is plagued by several challenges, including:

- The prevalence of **fake and substandard drugs**,
- The **arbitrary sale of medicines in open markets**,
- The **high cost of pharmaceuticals**, and
- **Inadequate enforcement mechanisms**, particularly the shortage of pharmaceutical inspectors.

Since pharmacy practice directly concerns the **availability, safety, efficacy, and authenticity** of pharmaceutical products, the **Pharmacists Council of Nigeria (PCN)** should naturally play the central regulatory role. However, overlaps in legal mandates have created systemic weaknesses.

5.2 Weaknesses in Law Enforcement

Although Nigeria has numerous laws regulating pharmacy practice, **many are outdated**, and implementation remains the greatest challenge. For example:

- Under the **Poisons and Pharmacy Act (Cap. 535 LFN)**, pharmaceutical inspectors lack prosecutorial powers. They must hand offenders over to the **police** for prosecution. In practice, offenders are often released, or penalties imposed are too weak to serve as deterrents.
- Many penalties under existing laws are **ridiculously low** compared to the severity of offences, creating little incentive for compliance.

5.3 Task Forces and Regulatory Conflicts

Despite multiple legislations against counterfeit drugs and narcotics, the problem persists due to **conflicts in enforcement structures**.

- The **Counterfeit and Fake Drugs (Miscellaneous Provisions) Decree No. 17 of 1997**, now Cap. 73 LFN, stipulates that the **Chairman of the Federal Task Force** should be an **Assistant Director in the Federal Ministry of Health**. In practice, however, the role is occupied by a **NAFDAC official**, raising legal questions that could invalidate prosecutions in court.

- The **Federal Ministry of Health** has gradually lost effective **supervisory control**, weakening coordination among enforcement bodies.
- Task Forces—originally meant to **dismantle illegal drug premises**—often focus instead on **raiding registered pharmaceutical premises**, while illegal outlets continue to flourish.
- Regulatory overlap also arises between **NAFDAC, NDLEA, and PCN**, as their enabling laws permit them to inspect pharmaceutical premises. Yet, under the law, only the **PCN is empowered to license such premises**.

This duplication of mandates creates **confusion, conflict, and inefficiency**.

5.4 Drug Distribution Challenges

Nigeria's drug distribution system remains **chaotic** and, at times, an embarrassment at the international level. In the past, certain **West African countries outrightly banned Nigerian pharmaceutical imports** due to concerns over quality and distribution practices.

5.5 Patent and Proprietary Medicine Vendors (PPMVs)

Another loophole exists in the **regulation of Patent and Proprietary Medicine Vendors (PPMVs)**:

- By law, PPMVs are only permitted to handle drugs in their **original, sealed packs**.
- In practice, many operate far beyond this legal scope.
- Nevertheless, recent regulatory measures by the **PCN** are beginning to achieve **modest success** in improving compliance within this category of medicine vendors.

6.0 Efforts Made to Review the Laws

The **Pharmacists Council of Nigeria (PCN)** has initiated a draft proposal to amend the existing **Pharmacy Law**, particularly **Act No. 91 of 1992**. This draft has been submitted to the **Legal Department of the Federal Ministry of Health** for review and subsequent transmission to the **Federal Ministry of Justice**.

6.1 Salient Provisions of the Proposed Amendments

The proposed amended law seeks to modernize and strengthen pharmacy regulation in Nigeria. Key provisions include:

1. Enhanced Regulatory Powers for PCN

- o Broader authority to monitor, regulate, and enforce compliance across all areas of pharmacy practice.
- o Clearer delineation of roles to avoid overlap with other regulatory bodies.

2. Strengthening of Inspection and Enforcement

- o Empowerment of **pharmaceutical inspectors** with prosecutorial powers.
- o Increased penalties for offences such as sale of counterfeit, substandard, or unregistered drugs, making them **more deterrent and proportionate** to the crime.

3. Reform of Drug Distribution Channels

- o Establishment of a **structured and transparent drug distribution system**, addressing the chaotic and unregulated markets currently in existence.
- o Stronger legal backing for the regulation of **Patent and Proprietary Medicine Vendors (PPMVs)**.

4. Professional Development and Re-Certification

- o Legal reinforcement of the **Mandatory Continuing Professional Development (MCPD)** programme for pharmacists as a requirement for license renewal.
- o Institutionalization of periodic training to ensure pharmacists remain updated with global best practices.

5. Alignment with International Standards

- o Harmonization of Nigerian pharmacy laws with **World Health Organization (WHO)** recommendations and global pharmaceutical practices.
- o Facilitation of **reciprocity agreements** for Nigerian pharmacists practicing abroad, ensuring recognition of professional qualifications.

S/N	PCN Decree 91 of 1992	New PCN Act 2004
1.	Provisions relating to the establishment of the Council and composition including financial provisions.	Retained in the proposed draft
2.	(a). Section 3(1) - on composition of Council paragraph (10(1) provided for one representative of the armed forces which shall be a registered Pharmacist; (b). Registrar not a member	a. Section 2 (1) (F) - the representative shall not be below the rank of a Colonel; b. The Registrar of the Council is made a member of Council; c. provided for one member representing Pharmaceutical manufacturers .
3.	Section 8 (3) - Registrar to keep register of Associate members, and premises.	Section 7 (c) (d) - added two registers for Pharmacy Technicians and Licensed Patent Medicine Vendors .
4.	Non-existent	Section 10 - introduces procedures for resolving registration matters between the Registry and applicants for registration.

5.	Non-existent	Section 10 (13) restrict the operation of retail pharmacy to pharmacist registered under the Act. Makes it mandatory for pharmaceutical companies for purposes of importation, exportation or wholesale to have a registered pharmacist on the Board of Directors and empowers the Council to refuse registration of premises if public health would be prejudice.
6.	Non-existent	Section 11- imported from Poison and Pharmacy Act the requirement for a Pharmacist to maintain direct and continuous personal control and management of the premises he superintends.
7.	Non-existent	Section 12 and 13- empowers authorized persons (Pharmaceutical Inspectors) to enter any place where drugs are sold, etc, and conduct investigation and take samples of suspected items.
8.	Section 14 (2) - a pharmacist with at least fifty (50) years post-registration experience shall not pay practicing fees.	Section 18 (3)- any Pharmacist with at least forty (40) years post-registration experience or has attained the age of sixty (60) years is exempted from the payment of retention fees to maintain name in the Register.
.	Section 14 (6) - Non-payment of practicing fee in a year attracts a punishment in the first instance to a fine twice the prescribed fees and ten times for a second or subsequent offence.	Section 18 (4) - raised the fine to 50% of the applicable fee within the year.
10.	Non-existent	Section 19- 21 - makes provisions for internee Pharmacists, approval of

		institutions for internship training and regulation of the practice by internee pharmacists.
11.	Section 23 - provided punishment for filling false statement with the Council to a fine not exceeding N1,000,00 on summary conviction and not exceeding N1 ,000.00 on indictment or exceeding 2 years imprisonment.	Section 28 - raised the fine in both to N20,000.00 and N50,000.00 respectively.
12.	Section 22 - makes provisions for library facilities only.	Section 29 - makes provision for Council to maintain library and laboratory facilities.
13.	Non-existent	Section 30 - provided against the obstruction of a Pharmaceutical Inspector.
14.	Non-existent	Section 31 - empowers the Council to prosecute offenders in a competent court of law.
15.	Non-existent	Section 33 - created an omnibus punishment provision on persons found liable open conviction to a fine of N50,000.00 or one year imprisonment or both.
16.	Non-existent	Section 34 - provided for training and registration of Pharmacy technician in Schools of Health Technology approved by the Council.
17.	Non-existent	Section 35 - empowers the Council to make guidelines for the Registration of Hospitals Pharmacies.
18.	Non-existent	Section 38 - makes provisions for re-

		certification before retention of names of registered persons, pharmacy technicians and sellers of medicine.
19.	Non-existent	Section 40 - provides for Pharmacist to offer First Aid Services where it is not reasonably practicable for the patient to consult a medical practitioner.
20.	Non-existent	Section 41 - provided for Mandatory sixty (6) days pre-litigation notice to be served on Council by a person who intends to commence a suit against the Council.
21.	Non-existent	Section 46 - 53 - made provisions for supply of restricted or dangerous.
22	Schedule 1 - prescribes minimum qualification to membership of Council to be not less than seven (7) years post-registration experience.	Schedule 1 - Raised the minimum qualification to ten (10) years.
23	Schedule 2 - the minimum qualification to become Assessor to the PCN Disciplinary Tribunal is seven (7) years standing.	Schedule 2 - raised the Assessors qualification to not less than ten (10) years standing.

7.0 Conclusion

The registration and/or retention of names of pharmacists—whether foreign-trained or Nigerian-trained but resident outside the country, particularly in the USA—is both feasible and fully accommodated within the extant laws. Under

Act 91 of 1992, the Registry is empowered to recommend to Council the appropriate guidelines on registration and reciprocity for members of NAPPSA and other registered pharmacists who wish to retain their names in the PCN Register.

At present, the Registry has engaged the services of a computer programmer to upgrade its systems and enable online annual license renewal for pharmacists. Once the Council formally approves the modalities for reciprocity, the Registry will immediately communicate these details to NAPPSA. For more information about the PCN, our official website www.pcnng.org can be visited.

In conclusion, I once again congratulate the leadership of NAPPSA for extending this invitation and for providing me with the opportunity to share my thoughts with colleagues in the United States. With modern information and communication technology, the barriers between nations are rapidly narrowing. Likewise, the “divide” between Nigerian pharmacists abroad and their responsibilities to their regulatory body—the PCN—is being bridged for the convenience and benefit of all.

Thank you and God bless.

Pharm. Ahmed T. Mora, B.Sc. (Pharm), MBA, FPSN, FPCPharm
Registrar, Pharmacists Council of Nigeria
Houston, Texas, USA

Sunday, September 16, 2007

✉ tijjanimora@yahoo.com | pcnig@yahoo.ca

BRIEF HISTORY OF PHARMACY LAWS

S/No	Ordinance/Law	Year	Purpose
1.	Lagos Pilotage and Harbour Ordinance	1978	Control the supervision of the applications of medicines and medical treatment on board ships docked in Lagos harbor.
2.	Hospital Ordinance	1881	
3.	Ereko Dispensary Rules	1889	
4.	Pharmacy Ordinance No. 8	1902	Control the sale and distribution of drugs and Poisons in Lagos, Calabar, Opobo, Warri Forcados or such as the Governor in Council deem fit.
5.	Poison and Pharmacy Ordinance	1925	Enacted when the Medical College, Yaba was established in 1925. It was enacted to fortify the registration of dispensers, trained chemists and druggist.
6.	Poison and Pharmacy Ordinance of 1936	1936	
7.	Poison and Pharmacy Act, Cap 152	1958	To amend the 1936 Ordinance
8.	Pharmacy Act Cap 26	1964	Partially repealed the Poison and Pharmacy Act 1958.
9.	LFRN Cap 357	1990	Retention of 1964 Act
10.	Pharmacist Council of Nigeria	Decree 92 of 1992	Section 26 (1) of this Decree repealed Cap 357.

RELATED LAWS AFFECTING THE PHARMACY PROFESSION

S/No	Law	Year	Emphasis
1.	Food and Drugs Act Cap 150 LFRN	1990	Makes provisions for the control of sale, manufacturing, distribution, advertisement, importation and exportation of food, drugs, cosmetics and devices. Issuances of certificate to manufacture vested to Honourable Minister of Health who can delegate. Condition is that, this must conform with the Pharmacists Act i.e. Superintendent Pharmacist as in the Cap 152.
2	Decree No. 21	1999	Amended, the FDA Sections 7, 9 and 17 address the issues of manufacturing prohibition by unauthorized persons; designation of inspectors, officer and analyst on recommendation of Food and Drug Advisory Council and imposition of penalty of fine not exceeding N1 ,000 or a term of imprisonment not exceeding 3 years or both respectively.
3	NAFDAC Decree No. 15 Of 1993	1993 but retrospective October 01, 1992	Section 28 - makes NAFDAC successor Agency to former Food and Drug Administration and Control Department of Federal Ministry of Heath.
			Section 5 - Provide the Agency the responsibility to regulate and control the important, exportation, manufacture, distribution and use of drugs, cosmetics, foods, water, chemicals, medicals devices.

			Section 27 - allows the Honourable Minister of Health to give directives of a special character and the Agency must comply. Section 24 - empowers officers of the Agency to enter premises to enforce provision of the Decree.
4	NDLEA Decree 33 Cap 253, 1990 LFRN	1990	Authority to deal with narcotics issues in the country.
5	NDLEA Decree (Amended) by Decree 15 of 1992	1992	This amendment gives all police powers, privileges and authorities to the Agency to enforce the provision of the enabling Decree. Section 3 - enforce the Agency to collaborate with Government bodies carrying on functions wholly or in part analogous to those of the Agency.
6	Poison and Pharmacy Act Cap 535, 2990 LFRN	1990	This Act regulates the sale and distribution of drugs and poisons. The PCN Decree 91 of 1992 is charged with the enforcement of the provisions of this Act.
7	Drug and Related Products (Registration, etc) Decree 1993.	1993	Promulgated principally to exhibit the manufacture, importation, exportation, advertisement, sale or distribution in Nigeria, of unregistered drugs, cosmetics and medical devices. NAFDAC enforces the provision of this law through the Drug Registration Committee.
8	Drug and Related Products (Registration, etc) Amendment	1999	Amended the 1993 Decree
9	Dangerous Drug Act Cap 91, 1990 LFRN		Section 7 (1) - empowers the President to make regulations to control the handling of cocaine and other drugs while item (2) provides for the

			authorization of any person running a Pharmacy in accordance with the provisions of the Pharmacy Act (Now Cap 535) to manufacture, compound and carry on the business of dispensing drugs. Section 17 (1) and (2) - empowers any Police Officer or pursuant to a warrant issued by a magistrate to enter premises of any person carrying on business related to this Act to inspect any document or stock of any such drugs if there are reasonable suspicion
.			of the commission of an offence and to seize and detain those drugs or documents as the case may be. Section 19 - makes provisions for penalties. The maximum to be meted against a defaulter is N200.00 fine or imprisonment for 2 years and/or forfeiture.
10	National Drug Formulary and Essential Drug List Cap 252, 1990, LFRN	1990	This Act prescribes a National Drug Formulary Essential Drug List and prohibits the importation into and manufacture in Nigeria of any drug not in the list save as shall be permitted by the Honourable Minister of Health under Section 3 on certain grounds including that the drug is a cure for any uncommon disease or diseases in requiring specialized skill for diagnosis and treatment, etc. There is also to be Review Committee to regulate dealings in drugs. Section 9 - provides for the establishment of a Secretariat or the Department of FDAC of the FMOH which shall be responsible for monitoring, and implication of the list. Section 8 - provides for penalty against a guilty person who may be liable to a fine of N1 ,000.00 or imprisonment to a term not

			exceeding 5 years.
	Counterfeit and Fake Drugs and Wholesome Processed Foods (Miscellaneous Provisions) Decree No.25, 1999	1999	This law prohibits the sale, production, importation, manufacturing, distribution, etc of counterfeit and fake drugs and unwholesome processed foods or sale of drugs or poisons in certain premises without duly licensed by the appropriate authority, defined as any person or body authorized to grant, license or register premises for sale or distribution of drugs or poisons. Section 6 - of this law establishes a Federal Task Force working in conjunction with the Nigeria Police Squad established under Section 10 to enforce the provision of the law. A person who commits an offence under Section 1 is liable on conviction to a fine not exceeding N500,000.00 or imprisonment for a term not less than 5 years or more than 15 years or more.

TEXT OF PAPER PRESENTED BY PHARM. AHMED T. MORA, FPSN, FPCPharm

THE REGISTRAR, PHARMACISTS COUNCIL OF NIGERIA (PCN)

**ON THE OCCASION OF THE FORMAL INAUGURATION OF THE DRUG
DISTRIBUTION INSPECTION COMMITTEE (DDIC) OF THE PCN
AT THE FRED ADENIKA AUDITORIUM ON WEDNESDAY, JULY 02, 2008**

Protocol

1.0 Introduction

It is my pleasure to welcome you all to this inauguration ceremony. Today marks the introduction of a new concept in the implementation of our regulatory and control functions under **Decree (now Act) 91 of 1992**.

We are all aware of the inspections carried out by Pharmaceutical Inspectors—either as staff of the PCN or under the Pharmaceutical Inspection Committee (PIC) of the Council. For clarity, inspections of premises by Pharmaceutical Inspectors, which may lead to approval and licensing of outlets where pharmaceutical activities take place, include:

- **Pharmaceutical Inspection Committee (PIC):** Retail, Wholesale, and Importation outlets.
- **Registry Staff (Inspection and Monitoring):** Pharmaceutical Manufacturing Companies.
- **Registry (Pharmacy Practice):** Hospital Pharmacies.

These inspections are not only for approvals and licensing but may also take the following forms:

Types of Inspections

1. **Comprehensive/Routine Inspection** – Generally reserved for:
 - o New pharmaceutical establishments;
 - o Establishments applying for permits to extend their scope of operation;

- o Establishments with major changes in key personnel or premises;
- o Establishments not inspected for 3–5 years;
- o Cases where credible information (even informal) suggests serious lapses.

Such inspections should be **announced**.

2. **Concise Inspection** – For establishments already inspected, aimed at assessing standards of good pharmacy practice. May be **announced**.
3. **Follow-up Inspection** – To verify that corrective measures previously recommended have been implemented. Where deadlines were given, this may be **unannounced**.
4. **Special Inspection** – Triggered by specific complaints of malpractice or non-compliance. Preferably **unannounced**.
5. **Investigative Inspection** – Used to assess the performance of new establishments whose scope of operation was previously unknown.

While inspections are generally announced, unannounced visits are necessary in some cases, especially **follow-up, special, and investigative inspections**.

Frequency of Inspections:

- All premises should be inspected at least **every 12–24 months**.
- Premises with frequent contraventions may be inspected **every six months**.
- Well-compliant premises may be inspected less frequently.

Most of us here have participated in one or more of these inspection types and have also received training—whether in-house, in external facilities, or in structured workshops.

2.0 Inspection of Establishments in the Drug Distribution Chain

The welfare of patients and the general public is of paramount concern in the drug distribution

chain, whether the drugs are manufactured locally or imported. Inspections are therefore undertaken to ensure:

- Protection of patients and the public from malpractices by distributors and suppliers;
- Compliance with drug laws and regulations on compounding, distribution, importation, exportation, and storage;
- Maintenance of high ethical and professional standards in pharmaceutical practice.

In this regard, **Section 5 of the Federal Republic of Nigeria Official Gazette (Lagos, 12th August, 2005; No. 81, Vol. 94)** provides guidelines under “*Proper Record Keeping in Premises*”:

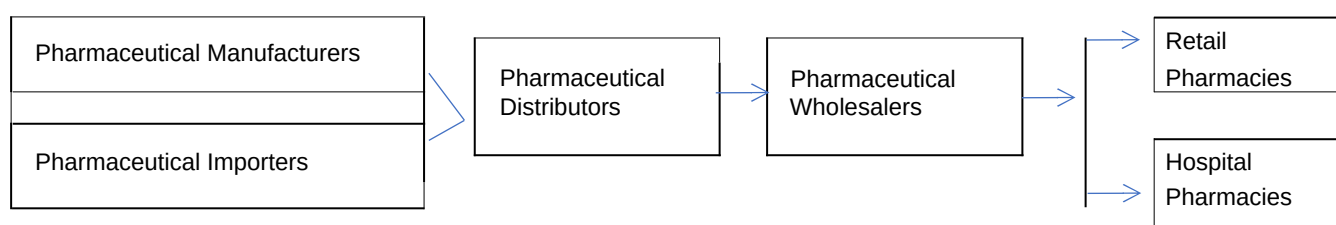
1. Manufacturers, importers, wholesalers, and retailers of drugs, poisons, and devices shall maintain appropriate records of the receipt and disposal of drugs.
2. Unauthorized disposal of drugs shall attract penalties.
3. Retail outlets shall keep records of:
 - o All purchases;
 - o Disposal of poisons;
 - o Ledgers/bin cards;
 - o Dispensed prescriptions.
4. Wholesalers/Distributors shall keep similar records as required of retailers.
5. Importers shall maintain:
 - o Relevant import documents;
 - o Records as required under subsection (3).
6. Manufacturers shall keep records relating to:
 - o Purchases of raw materials;

- o Production and quality control activities;
- o Distribution and sales of finished products.

From the above, it is evident that the **PCN is embarking on a new and broader form of inspection**—different from the traditional inspections specific to particular practice areas.

In this **Drug Distribution Inspection**, the focus is on the **entire drug distribution chain**. In essence, the chain as a whole—not just individual segments—is under scrutiny.

A typical distribution chain may follow this flow:



This type of inspection addresses the multiplicity of practices as a system. It entails a thorough and painstaking as well as meticulous approach. In a manner, it could be investigative, as once a trail has been identified, the tracking must continue till the end, which is at the retail or hospital pharmacy levels.

3.0 Drug Manufacture, Dispensing, and Medication Errors

A **medication error** is defined as any **preventable event** that may cause or lead to inappropriate medication use or patient harm while the medication is under the control of a healthcare professional, patient, or consumer. Such events can arise from issues related to:

- Prescribing,
- Order communication,
- Product labeling, packaging, or nomenclature,

- Compounding,
- Dispensing,
- Distribution,
- Administration,
- Patient education, monitoring, and use.

In the specific context of pharmacy practice, **medication errors** often occur within healthcare facilities—such as hospitals, retail pharmacies, and manufacturing settings—where pharmacists play a pivotal role.

Perspectives from Literature

According to Ghanaian pharmacists **H. K. Abutiate and Edward O. Amporful**, in their paper “*Medication Errors Prevention: A Pillar for Patient Safety*” (The African Pharmacist, Vol. 1, No. 1, 2008), medication errors are incidents in which there has been an error in prescribing, dispensing, preparing, administering, monitoring, or providing medicine advice—**regardless of whether harm occurred or not**.

They further stated that in **England**, preventable harm from medicines costs an estimated **£750 million annually**. This underscores the economic and human burden of medication errors.

The **College of Nurses of Ontario (2006) Medication Practice Standard** emphasized preventing such errors by adhering to the **Five Rights of Medication**

Use:

1. The **right drug**,
2. The **right dose**,
3. The **right patient**,
4. The **right time**,

5. The **right documentation (or note)**.

Similarly, **William A. Zellmer**, in his book “*The Conscience of a Pharmacist: Essays on Vision and Leadership for a Profession*” (ASHP, 2002), argued that pharmacists—particularly hospital pharmacists—must play a leadership role in preventing medication errors. He identified three critical “planes” of intervention:

- **First Plane – Individual Competence** Pharmacists must recognize potential lapses in drug preparation, distribution, and administration, and build appropriate checks into their daily practice.
- **Second Plane – Hospital Systems** Hospitals must design their medication systems (ordering, dispensing, and administering) to minimize human error.
- **Third Plane – Quality Assurance** Pharmacy departments should implement continuous monitoring systems for detecting and reducing dispensing and administration errors.

Zellmer concluded that reducing medication errors requires **vigilance at every level of healthcare**, with pharmacists leading multidisciplinary programmes designed to prevent errors.

The Critical Role of Pharmacists

The **pharmacist’s contribution** is central at every stage of the drug-use process:

- **Manufacturing:** Ensuring pharmaceutical products are formulated correctly.
- **Dispensing:** Providing the right medicine to the right patient with proper labeling.

- **Patient Counseling:** Giving clear, precise, and unambiguous instructions for use.

Even a well-formulated and properly labeled medicine can become ineffective—or even harmful—if handled or dispensed unprofessionally. Thus, **the pharmacist stands as the decisive factor** in whether a medicine is used safely and effectively.

The **World Health Organization (WHO)**, in its *2008 Summary of the Evidence on Patient Safety*, highlighted that including pharmacists in hospital medical teams significantly reduces errors. A 1999 study showed that having a pharmacist on medical wards reduced **medication ordering errors by 66%**, from 10.4 to 3.5 per 1,000 patient days.

Clearly, the **pharmacist, the premises, and the practice environment** are all critical in minimizing medication errors.

4.0 The Challenge for the Committee

This Committee faces very challenging tasks for several reasons.

First, there is no documented guideline or Standard Operating Procedure (SOP) for this type of inspection in Nigeria. The Committee, therefore, has to develop its own working system.

Second, inspections may be frustratingly long, as tracking a particular product—especially suspicious ones—could take considerable time.

Third, invoices, receipts, and other industry documents may be deliberately hidden or falsified by outlets in an attempt to mislead the Committee's work.

Fourth, Committee members should also expect the deliberate creation of dead ends by some outlets.

Nonetheless, I am confident that the members of this Committee are seasoned professionals who will rise to the challenge. While it is expected that much of the

Committee's work will take place in Lagos, it may not end there.

For effectiveness, the Committee must equip itself with the following during inspections:

- A list of all registered premises in the country for the current year, and at least the two preceding years.
- Calculators, notebooks, and other tools for proper documentation, including the recording of invoice and receipt numbers.

I recommend that Committee members use today and tomorrow to define the modus operandi for their assignment, request all necessary materials, and set clear procedures.

It is not necessary for the entire Committee to be present at every outing. I strongly recommend that the Committee be split into two groups, with each group taking turns to conduct inspections and later convening to share findings. This will ensure manageable numbers at each facility and prevent disruption of Registry work on a larger scale across the various offices from which members are drawn.

On this note, I hereby inaugurate the Drug Distribution Inspection Committee (DDIC) for the inspection of facilities, as outlined under Decree (now Act) 91 of 1992 and in accordance with Regulation 81, Vol. 904 of the Federal Republic of Nigeria Official Gazette, Lagos, dated August 12, 2005.

Thank you all, and good luck.

Pharm. Ahmed T. Mora, FPSN, FPCPharm
Registrar

TEXT OF PRESENTATION

BY

PHARM. AHMED T. MORA, FPSN, FPCPharm

Registrar/CEO, Pharmacists Council of Nigeria (PCN), Abuja, Nigeria

ON THE OCCASION OF THE

Second Annual Convention of the Nigerian Association of Pharmacists and
Pharmaceutical Scientists in the Americas (NAPPSA)

AT

Atlanta, Georgia, USA

SEPTEMBER 18 – 20, 2008

**THE NIGERIAN PHARMACIST, MEDICATION ERRORS, AND THE
REGULATION AND CONTROL OF PHARMACY PRACTICE BY THE
PHARMACIST’S COUNCIL OF NIGERIA (PCN)**

By:

Pharm. Ahmed T. Mora, FPSN, FPCPharm

1.0 Introduction

As the battle against fake, counterfeit, and substandard drugs rages on, the need for an integrated and multidisciplinary effort to win the fight becomes even more

apparent. It is the responsibility of every healthcare professional to ensure that medications and drugs intended for prophylactic, curative, and/or diagnostic purposes within healthcare facilities are genuine, safe, and efficacious.

A **pharmacist** is a person professionally qualified in **pharmacy**, the branch of health sciences dealing with the preparation, dispensing, and rational use of medicines. Globally recognized and licensed, the pharmacist prepares, compounds, mixes, and dispenses medicines for the benefit of patients and the healthcare delivery system.

The pharmacist's role in ensuring the successful use of medicines for treatment is total and paramount among healthcare professionals. Put simply, no matter the accuracy of diagnosis, the soundness of prescriptions, or the genuineness of medicines prescribed, if the pharmacist lacks professional competence due to inadequate updates or knowledge, medication errors are most likely to occur.

It is therefore incumbent on the **regulatory body**—the Pharmacists Council of Nigeria (PCN)—to ensure that only fit, proper, and competent pharmacists are licensed to practice. Pharmacy practice traditionally takes place in hospital pharmacies, retail pharmacies, importation/wholesale firms, and pharmaceutical manufacturing industries. The PCN registers, inspects, approves, and licenses these premises in line with **Act 91 of 1992**.

2.0 The Nigerian Pharmacist

The Nigerian pharmacist is a healthcare professional registered by the PCN to practice pharmacy in Nigeria. He or she must:

- Graduate from any of the approved Faculties of Pharmacy in Nigerian universities;
- Complete internship training in an accredited centre; and
- Obtain an annual license from the PCN to practice.

Alternatively, foreign-trained pharmacists with at least a Bachelor's degree in Pharmacy must pass the **Mandatory Orientation Programme for Foreign-Trained Pharmacists** at a PCN-approved centre before being licensed.

Ensuring continuous competence and fitness to practice is a basic responsibility of the PCN. This is largely achieved through **Mandatory Continuing Professional Development (MCPD)** programmes in accredited centres. MCPD exposes pharmacists to current developments in pharmaceutical sciences, thereby improving their knowledge and skills for the benefit of patients.

The ultimate objective is to present well-trained, registered, and currently licensed pharmacists to the public—professionals who dispense appropriate pharmaceutical products and services within PCN-registered, inspected, approved, and licensed premises.

To enforce compliance, the PCN carries out regular inspections—both announced and unannounced. Depending on inspection reports, a pharmacy may:

- Continue to offer services,
- Be partially restricted, or
- Be sealed outright.

These measures are to ensure that the public accesses only those premises that meet PCN standards, as mandated by Act 91 of 1992.

3.0 Drug Manufacture, Dispensing, and Medication Errors

A **medication error** is any preventable event that may cause or lead to inappropriate medication use or patient harm while the medication is under the control of a healthcare professional or patient. Errors may arise from professional practice, healthcare products, procedures, and systems, including:

- Prescribing,
- Order communication,
- Product labeling and packaging,
- Compounding,
- Dispensing,
- Distribution,
- Administration,
- Patient education, monitoring, and use.

Abutiate and Amporful (2008), in *The African Pharmacist*, defined medication errors as incidents in which errors occur during prescribing, dispensing, preparation, administration, monitoring, or advice—whether or not harm occurs. They estimated that preventable harm from medicines costs England over **£750 million annually**.

Medication errors begin at the **prescription stage**, which can lead to downstream problems. The **College of Nurses of Ontario (2006) Medication Practice Standard** highlights human factors in errors and prescribes the five “rights”:

- Right drug,
- Right dose,
- Right client,

- Right time, and
- Right note.

William A. Zellmer (2002), in *The Conscience of a Pharmacist*, stressed that pharmacists must do all they can to prevent medication errors, particularly in hospitals where they occur more frequently. He identified three levels of action:

1. **Personal vigilance** – recognizing lapses in preparation, distribution, and administration, and creating effective checks.
2. **Hospital systems** – designing medication processes that minimize human error.
3. **Quality assurance** – establishing ongoing systems to monitor and evaluate dispensing and administration errors.

He concluded that reducing errors requires vigilance at multiple levels, with pharmacists taking the lead in multidisciplinary programs.

Pharmacists' skills in formulation, dispensing, and patient counseling significantly reduce errors. Every pharmaceutical product carries vital information, and only pharmacists are trained to interpret and communicate this information professionally. Counseling must be clear, precise, and unambiguous to ensure compliance.

MCPD participation is therefore not optional; failure to comply results in non-licensure, regardless of prior registration.

The **WHO (2008) Summary on Patient Safety** emphasized the pharmacist's role, citing a study in which the presence of a pharmacist reduced medication ordering errors by **66%**.

Thus, the pharmacist, the premises, and the facilities are all critical in ensuring patient safety.

4.0 Setting Standards for Pharmacy Practice by the PCN

Pharmacy practice in Nigeria has a long history:

- The first outlet opened in **1887** at Balogun Street, Lagos.
- The **Pilotage and Harbor Ordinance (1878)** and the **Hospital Ordinance (1881)** laid early foundations.
- The **Pharmacy Ordinance No. 8 of 1902** formally regulated the practice.
- The **Poison and Pharmacy Ordinance (1923)** and subsequent establishment of the **Board of Medical Examiners (1927)** further shaped regulation.

From 1927 to 1992, successive bodies—the Board of Medical Examiners, the Pharmacy Board of Nigeria, and the Pharmacists Board of Nigeria—regulated pharmacy until **Decree 91 of 1992 (now Act 91)** established the **PCN**.

The PCN regulates pharmacy in all aspects. Section 26 of the Act transferred all powers of the defunct PBN to the Council. Courts have upheld its authority to regulate both public and private pharmacy practice.

On **August 16, 2005**, the PCN launched the **Four-Part Compendium of Minimum Standards for the Assurance of Pharmaceutical Care**, defining the boundaries and scope of practice in hospital, retail, wholesale/importation, and manufacturing pharmacy. This historic publication, presented by Hon. Minister Prof. Eyitayo Lambo, positioned PCN as a proactive and credible regulatory authority.

The **Compendium** is now a required reference material in all pharmaceutical premises before registration or licensing.

5.0 Conclusion

The pharmacist plays a central role in ensuring patient compliance and reducing medication errors. Within the healthcare delivery chain, the professional responsibility for producing, compounding, and dispensing medicines rests squarely on the pharmacist.

The late U.S. President **John F. Kennedy** once remarked:

“The pharmacist is an indispensable link in the chain of national health protection. If we did not have the pharmacist, it would be necessary to invent him.”
(*Pharmacy Practice Management in Developing Communities*, Fred B. Adenika, 1975).

Today, Nigerian pharmacists stand tall among their healthcare colleagues. The PCN’s role in training, equipping, and licensing competent, fit, and proper pharmacists is vital. By enforcing standards, accrediting premises, and mandating MCPD, the PCN ensures the public receives the **highest quality pharmaceutical care**.

Pharm. Ahmed T. Mora, FPSN, FPCPharm
Registrar, PCN
September 18–20, 2008